



File No.: EXP202408617

RESOLUTION OF RIGHTS PROCEDURE

Having reviewed the complaint registered on May 26, 2024, with this Agency and having carried out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: On May 26, 2025, a complaint was submitted to this Agency by A.A.A. (hereinafter, the complainant) against the EXTREMADURA HEALTH SERVICE (SES) (hereinafter, the respondent) for not having duly attended to their right of access to the medical history of their minor child.

The complainant states that, on several occasions, they have requested access to the medical history of their minor child, having not received a satisfactory response, finding that certain documentation is missing, including: list of accesses to the medical history, prescribed medications, interconsultation reports, SOAP reports, requests for external consultations...

They provide a copy of the request submitted on April 23, 2024, in their capacity as legal guardian of their child, addressed to the MANAGER OF AREA PLASENCIA, in which, in summary, they indicate the following:

- That on April 1, 2024, they requested lists of access as of the date of submission of the request and prescribed medications as of ***DATE.2 (name of the medication, national code, date of prescription, and physician), and "(...) this last request was denied without any justification, as stated in the document for the withdrawal of the documentation."
- That on April 2, 2024, they requested everything related to the emergency visit dated ***DATE.1, "(...) and the interconsultation reports were not provided, as reflected in the document for the withdrawal of documentation."
- That on April 4, 2024, they reiterated the request for access documents to the medical history, the list of medications up to ***DATE.2, as well as the SOAP reports of pediatric consultations on dates ***DATE.3, ***DATE.4, and ***DATE.5 "(...) and again they were denied without justification, as we expressed in the document for the withdrawal of documentation."
- That on April 10, 2024, "Medical appointments were requested at the Hospital and/or external consultations between ***DATE.5 and ***DATE.6, whether or not they attended, as well as all interconsultation reports, along with the physician and the dates that request those consultations. Furthermore, the access history to the Patient's file by Physician B.B.B. as of the current date was requested."

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- April 17, 2024: They indicate that when they personally went to check the status of their request, the document was in the personal briefcase of the head of the administrative unit, who informed them that "the requests were pending management."

They also indicate the described request that "(...) Finally, the interconsultation reports and the physicians and the dates that request those consultations were not provided, as we reflected in the document for the withdrawal of documentation."

And they request:

1. A list of prescribed medications between the dates ***DATE.7 and ***DATE.2 (name of the medication, national code, date of prescription, and physician).
2. Regarding the pediatric consultation dated ***DATE.8 (C.C.C.) at Virgen del Puerto Hospital, the date of request for this consultation, the physician who requested it, the interconsultation report, and other data sent from the Health Center to request the aforementioned appointment.

3. In relation to the consultation ***DATE.9, not having been requested by the parents, they require to know when the request was made (date and time), from which medium (health center, phone, internet); in the case of having been requested electronically, the person who requested it, as well as their IP address; in the case of being by phone, from which phone; and in the case of being from a health center, to know which worker registered the appointment.

The complainant also adds in the complaint submitted to this Agency the possible violation of the principle of confidentiality and security measures, alleged unauthorized accesses to their child's medical history by a physician, and that dates of consultations have been subsequently modified and consultations have been fraudulently requested. They do not provide supporting documentation for the claims made.

Furthermore, the complainant has not provided a copy of the access requests mentioned in the last request that has been attached, and whose requests are reflected in the three points above.

SECOND: In accordance with Article 65.4 of the LOPDGDD, the complaint was forwarded to the respondent for analysis and to inform this Agency within one month of the actions taken to comply with the requirements set forth in data protection regulations.

The notification of the transfer of the complaint, which was carried out in accordance with the rules established in Law 39/2015, of October 1, on the Common Administrative Procedure of Public Administrations (hereinafter, LPACAP), was made on June 21, 2024, as stated in the acknowledgment of receipt issued by the Single Authorized Electronic Address Service (DEHÚ) that is part of the file. There is no record that any written response from the respondent has been received by this Agency in response to said transfer.

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THIRD: The result of the transfer procedure indicated in the previous Fact did not allow for the understanding that the claims of the complaining party were satisfied. Consequently, on August 26, 2024, for the purposes provided in Article 64.1 of the LOPDGDD, the Director of the Spanish Agency for Data Protection agreed to admit the complaint filed for processing.

The aforementioned agreement granted the complained party a hearing procedure, so that within a period of ten working days, they could present the allegations they deemed appropriate.

The complained party, in relation to the transfer of the complaint made from this Agency, indicates that "(...) there is no record of such communication. This is not intended to cast doubt on the procedures or to show a situation of defenselessness, but rather to indicate that if no response was received to said document, it is for no other reason than ignorance of it. (...)")

The complained party adds that, since the complaining party has not provided documentation evidencing the access requests and the lack of attention to them, "(...) it does not seem reasonable to continue a sanctioning procedure that is not based on proven facts or reasonable evidence, given that it risks generating an unwanted situation of defenselessness. Especially when the reported facts can be easily substantiated by the complainant and, moreover, the current situation does not prevent them from exercising their right of access that they claim has been denied."

Regarding the request of the complaining party, "Article 15 of Law 41/2002, of November 14, which regulates the autonomy of the patient and rights and obligations in the field of information and clinical documentation, establishes the minimum content of the clinical history, among which the information requested by the citizen is not included.

It should be noted that the request is made under the clinical history of the minor and, therefore, the rights regulated regarding it in the aforementioned Law 41/2002 (and in its autonomous development by Law 3/2005, of July 8, on health information and patient autonomy) apply. Thus, the request for information regarding the dates on which an appointment request was made cannot be accepted as it is not part of it.

Regarding the request for information related to a consultation dated ***DATE.9, the information requested far exceeds the right of access, whether exercised or not over the Clinical History. It is worth recalling at this point the recent ruling from January 2024 by the AN that upheld the AEPD's criterion, in that the citizen does not have an implicit right to know who, within a healthcare system, accesses their clinical history; it is true that, in Extremadura, Law 3/2005 establishes an implicit obligation for the SES by recognizing, in Article 35.3 of said regulation, the patient's right "to obtain copies or certificates of the aforementioned documents, and to know in any case who has accessed their health data, the reason for the access, and the use that has been made of them," but it does not establish any obligation to provide the information requested.

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to point out the technical difficulties that are required and even, as is the case, the impossibility of obtaining that information." Regarding the alleged unauthorized accesses and improper modification of dates referred to by the claimant, the respondent indicates that they have not provided any documentary evidence to substantiate those claims.

FOURTH: After examining the document submitted by the respondent, it was forwarded to the claimant, so that, within a period of ten working days, they could make any allegations they deemed appropriate.

The claimant submitted a document to this Agency dated November 18, 2024, providing copies of two emails sent on April 1, 2024, and April 4, 2024, to "Dirección Médica Atención Primaria del Área de Salud de Plasencia," dimed.plasencia@salud-juntaex.es, and their corresponding responses from the recipient indicating "Received." They state that the request dated April 2, 2024, was submitted at the counter, copies were not provided, and regarding the request dated April 17, 2024, "Digital format evidence AUDIO RECORDINGS will be provided upon request from AEPD, containing a conversation in which I participate."

The claimant reiterates, in summary, their requests for various information. They indicate that they request data related to the appointment of ***DATE.9 "(...) This data is requested since neither my wife nor I have requested this appointment; moreover, neither my wife, nor I, nor anyone has attended the consultation on the cited day with or without our child. It is obvious that someone has an interest in having us appear at the health center that day; therefore, there is a high risk that someone could be manipulating data, which poses a risk."

"Regarding unauthorized access: B.B.B., while on leave ***DATE.10 accessed my son's medical history as well as on the day ***DATE.11 based on a non-existent appointment, which is why we request all the information regarding that appointment, and during the year 2024, there have been accesses and supposed modifications of documentation."

FIFTH: After granting the respondent the opportunity to be heard, there is no record that any response has been received by this Agency.

LEGAL GROUNDS

I

Competence

In accordance with the powers granted to each supervisory authority by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and as established in Articles 47, 48.1, and 64.1 of the LOPDGDD, the Presidency of the Spanish Agency for Data Protection is competent to resolve this procedure.

Furthermore, Article 63.2 of the LOPDGDD states: "The procedures processed by the Spanish Agency for Data Protection shall be governed by the provisions of..."

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in Regulation (EU) 2016/679, in this organic law, by the regulatory provisions issued in its development and, to the extent that they do not contradict them, subsidiarily, by the general rules on administrative procedures."

II

Preliminary Issues

In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among data controllers and processors regarding their obligations, as well as addressing complaints filed by a data subject and investigating, as appropriate, the reasons for such complaints.

Correspondingly, Article 31 of the GDPR establishes the obligation of data controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In cases where they have designated a data protection officer, Article 39 of the GDPR assigns this officer the function of cooperating with said authority.

In accordance with this regulation, following the procedure established in Article 65.4 of the LOPDGDD, prior to the admission of the complaint regarding the exercise of the rights regulated in Articles 15 to 22 of the GDPR, which gives rise to this procedure, the complaint was forwarded to the respondent party for analysis, to respond to this Agency within one month, and to provide evidence of having given the complainant the appropriate response.

The result of this forwarding did not satisfy the claims of the complainant. Consequently, on August 26, 2024, for the purposes provided in Article 64.1 and 65 of the LOPDGDD, the complaint was admitted for processing. This admission initiates the present procedure regarding the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, which states:

"When the procedure refers exclusively to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it will begin with an admission agreement, which will be adopted in accordance with the provisions of Article 65 of this organic law.

In this case, the deadline for resolving the procedure will be six months from the date on which the admission agreement was notified to the complainant. After this period, the interested party may consider their complaint accepted."

Article 58.2 of the GDPR grants the Spanish Agency for Data Protection a series of corrective powers to address any infringement of the GDPR, including "ordering the data controller or processor to attend to the requests for the exercise of the rights of the data subject under this Regulation."

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III

Rights of Individuals Regarding Personal Data Protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights include access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, the provisions expressed in Recitals 59 and following of the GDPR are taken into account. In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the data subject, and must express its reasons in case it does not intend to comply with such request. The

burden of proof for compliance with the duty to respond to the request for the exercise of rights made by the affected individual lies with the data controller.

The communication addressed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

IV

Right of Access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the data subject has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data."

Regarding the right of access to personal data, according to Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the data controller may request the affected individual to specify the "data or processing activities to which the request refers." The right will be considered granted if the data controller provides remote access to the data, with the request being deemed fulfilled (although the data subject may request information regarding the aspects provided in Article 15 of the GDPR).

The exercise of this right may be considered repetitive if it is exercised more than once within a six-month period, unless there is a legitimate reason for doing so. On the other hand, the request will be considered excessive when the affected individual chooses a means other than the one offered that incurs a disproportionate cost, which must be borne by the affected individual.

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Like the other rights of the interested party, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the available information about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed shall not adversely affect the rights and freedoms of others.

The right of access in relation to the medical history is specifically regulated in Article 18 of Law 41/2002, of November 14, which is the basic regulation governing Patient Autonomy and Rights and Obligations regarding Information and Clinical Documentation (hereinafter LAP), the literal wording of which states:

"1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical history and to obtain a copy of the data contained therein. Health centers shall regulate the procedure that guarantees the observance of these rights.

2. The patient's right of access to the medical history may also be exercised by duly accredited representation.

3. The patient's right of access to the documentation of the medical history may not be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the right of the professionals involved in its preparation, who may oppose the right of access with the reservation of their subjective notes.

4. Health centers and individual practitioners shall only grant access to the medical history of deceased patients to persons linked to them, for family reasons or otherwise, unless the deceased expressly prohibited it and this is accredited. In any case, access by a third party to the medical history motivated by a risk to their health shall be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals shall not be provided, nor information that

harms third parties."

In this regard, it is important to highlight Article 15 of the LAP, which outlines the minimum content of the medical history:

"1. The medical history shall include information that is considered essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in the primary care and specialized care settings.

2. The main purpose of the medical history shall be to facilitate healthcare, recording all those data that, under medical criteria, allow for the truthful and updated knowledge of the health status.

The minimum content of the medical history shall be as follows:

- a) The documentation relating to the clinical-statistical sheet.
- b) The admission authorization.
- c) The emergency report.
- d) The anamnesis and physical examination.
- e) The evolution.
- f) The medical orders.
- g) The consultation sheet.
- h) The reports of complementary examinations.
- i) The informed consent.
- j) The anesthesia report.
- k) The operating room report or delivery record.
- l) The pathological anatomy report.
- m) The evolution and planning of nursing care.
- n) The therapeutic application of nursing.
- ñ) The chart of vital signs.
- o) The clinical discharge report.

Paragraphs b), c), i), j), k), l), n) and o) shall only be required in the completion of the medical history when it concerns hospitalization processes or as otherwise provided.

3. The completion of the medical history, in aspects related to direct patient care, shall be the responsibility of the professionals involved in it.

4. The medical history shall be maintained with criteria of unity and integration, in each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the professionals of the data of a particular patient in each care process."

Regarding the preservation of the medical history, Article 17 of the LAP, in its points 1 and 5, states that:

"1. Health centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original support, for the proper assistance to the patient for the appropriate time in each case and, at a minimum, five years from the date of discharge from each care process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate."

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Regarding the autonomous community, Article 4 of Law 3/2005, of July 8, on health information and patient autonomy, provides for the right to health information, which establishes the following:

1. Patients in the Autonomous Community of Extremadura have the right, in relation to any action concerning their health, to receive all available information regarding it, except in cases exempted by this regulation.

2. The information must refer to all health care actions, constituting a fundamental part of them, including diagnosis, prognosis, and therapeutic alternatives, and must cover at least the purpose and

nature of the action, as well as its risks and consequences.

3. Furthermore, the information provided to the patient must include the name, qualifications, and specialty of the health professionals attending to them, as well as their category and function, if defined in their center or institution.

4. As a general rule, the information will be provided verbally, with a record made in the clinical history. It will be truthful, provided in a comprehensible manner and appropriate to the needs and requirements of the patient, with sufficient advance notice, to assist them in making decisions in accordance with their own free will.

5. Every person has the right to have their wish to not be informed respected, and to not have information about their health status or illness transmitted to individuals related to them by family ties or otherwise, nor to third parties, by expressing this in writing. The written waiver must be incorporated into the clinical history. This right will not be recognized when there is a high risk of transmission of a serious illness, and such circumstances must be justified in the clinical history.

V

Conclusion

The rights procedure is initiated as a consequence of the lack of attention to any of the rights regulated in the data protection regulations. Therefore, in this case, only the facts related to the exercise of these rights will be analyzed and evaluated, excluding the other issues raised by the parties.

In this case, and considering the provisions of the first paragraph of this Legal Basis, from the examination of the documentation provided, it has been established that the claimant requested access to the clinical history of their minor child, as well as various information about them, and that this access has not been addressed or has been denied with justification. This access request is documented, at least, in the request submitted on 04/23/2024 directed to the responding party and which accompanies the complaint.

Although the claimant has not provided the access requests to the clinical history mentioned in their complaint, a copy of the registration receipt for the aforementioned request submitted on 04/23/2024 is included in the file.

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The responding party has not provided any documentary evidence that would allow consideration that they have expressly responded to the claimant, fulfilling the provisions of Article 12.4 of the GDPR, addressing the right or denying it with justification. According to this article, it is the responsibility of the data controller to demonstrate that they have fulfilled their obligation to address the exercised right, either by providing the requested information or justifying its denial.

The aforementioned regulations do not allow the request to be overlooked as if it had not been made, leaving it without the response that the data controllers are obliged to issue, even in cases where there are no data in the files or even in those cases where the request does not meet the stipulated requirements, in which case the recipient of the request is also obliged to require the correction of the observed deficiencies or, if applicable, deny the request with justification indicating the reasons why the right in question cannot be considered.

Therefore, the request made obliges the controller to provide an express response in all cases, using any means that justifies the receipt of the response.

Since no copy of the necessary communication that must be directed to the claimant informing them about the decision made regarding the request for the exercise of rights is attached, it is appropriate to uphold the complaint that initiated this procedure.

In view of the cited provisions and other generally applicable regulations, the Presidency of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO UPHOLD the complaint made by A.A.A. considering that the provisions of Article 15 of the

GDPR have been infringed and to urge the EXTREMADURA HEALTH SERVICE (SES) with NIF Q0600413I, to send to the claimant, within ten working days following the notification of this resolution, a certification addressing the requested right or denying it with justification indicating the reasons why the request cannot be addressed, in accordance with the provisions of this resolution. The actions taken as a consequence of this Resolution must be communicated to this Agency within the same period. Non-compliance with this resolution could result in the commission of an infringement of Article 83.6 of the GDPR, classified as very serious for the purposes of prescription in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

SECOND: NOTIFY this resolution to A.A.A. and to the EXTREMADURA HEALTH SERVICE (SES). In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

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Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Presidency of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file an administrative contentious appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and Section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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Olga Pérez Sanjuán

The Deputy Director General of Data Inspection, in accordance with Article 48.2 LOPDGDD, due to the vacancy of the Presidency and Deputy Presidency

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